

HOW TO OPEN AN ASSISTED LIVING RESIDENCE

or Comprehensive Personal Care Home in New Jersey

Apartment-style ALR or homelike CPCH — licensed by the NJ Department of Health.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The New Jersey model — read this first

New Jersey regulates assisted living under one set of rules — N.J.A.C. 8:36 — that covers three related models, all licensed and surveyed by the New Jersey Department of Health, Division of Long-Term Care Systems. Which model fits your plan is the first and most important decision, because it shapes your building, your staffing, and how you get paid. This guide focuses on the two core ownership paths — the Assisted Living Residence (ALR) and the Comprehensive Personal Care Home (CPCH) — and is honest about the part everyone asks about: how you actually get paid.

Two things make New Jersey distinct — they run through everything. (1) Medication administration by delegation: trained, unlicensed staff may ADMINISTER medications to residents under the delegation and supervision of a registered professional nurse — not merely “assist with,” as in many states — which puts a full-time RN and a trained medication-aide workforce at the center of your operation. (2) Aging in place and managed risk: the rules expect you to help a resident stay as their needs grow, and to honor a resident's informed choices about risk through a written managed-risk agreement.

The three models, in one line. ALR = apartment-style units (private unit, bath, kitchenette, lockable door). CPCH = a homelike setting with bedrooms of no more than two residents behind a lockable door — the smaller-operator entry point, and what many former residential health care facilities and boarding homes convert into. ALP = assisted living services brought to tenants of publicly subsidized housing, rather than a facility that houses residents itself. This guide covers the ALR and CPCH.

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened a facility — every time we name a thing, we stop, break it down, and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; confirm every step, figure, and contact with the Department of Health (see Part 12) before you rely on it — fees, forms, and program numbers change.

The three models — what each one means

Model	What it is	Who pays
Assisted Living Residence (ALR)	Apartment-style units (private unit, bath, kitchenette, lockable door), coordinated services, aging in place. Requires a certificate of need.	Private pay, or Medicaid MLTSS for services + resident income/SSI for room & board
Comprehensive Personal Care Home (CPCH)	Homelike setting; bedrooms with no more than two residents per unit and a lockable door. The smaller-operator entry point.	Same as ALR — MLTSS for services + income/SSI for room & board
Assisted Living Program (ALP)	Assisted living services delivered to tenants of publicly subsidized housing, not a facility itself.	MLTSS for services; housing via the subsidized-housing program

The journey at a glance

Nine milestones take you from idea to a licensed, MLTSS-participating home.

1	Decide your model & set up the business	ALR (apartment-style) or CPCH (homelike); entity, EIN, NPI, insurance (Parts 3–4)
2	Get your Certificate of Need	The CON gate — expedited review is available for assisted living (Part 5)
3	Plan review & the physical plant	Submit construction plans to DCA before you build or renovate (Part 6)
4	Apply for the license	Application to DOH (\$1,500 + \$15/bed), admission agreement, pre-licensure survey (Part 7)
5	Recruit leadership	A licensed AL administrator and a registered professional nurse (Part 8)
6	Build the medication & care model	Trained medication aides under RN delegation; assessment & managed risk (Parts 8–9)
7	Set up how you get paid	Private pay + MLTSS for services; the room-and-board split (Part 10)
8	Plan your Medicaid-access commitment	Enroll in MLTSS and plan your resident mix from day one (Part 10)
9	Open & operate for surveys	Admit against your assessment process; report reportable events; renew annually (Part 11)

Rule of thumb. The Certificate of Need comes **FIRST** — do not sign a lease or start construction assuming you can license without it. And plan your Medicaid-access commitment from day one, not at month 30.

PART 1

What This Business Is

In short: A licensed assisted living operation — apartment-style (ALR) or homelike (CPCH) — where residents receive coordinated personal care, supervision, medication administration, and health-related services, and can age in place as their needs grow. Paid by private pay and, for services, Medicaid MLTSS.

An assisted living residence provides housing plus coordinated services to older adults and people with disabilities who need help with daily living but not the full-time skilled care of a nursing home. Residents live in their own unit, receive personal care and supervision, and — this is New Jersey's signature — can have medications administered by trained staff and can stay as their needs change rather than being moved out at the first decline. A CPCH delivers the same services in a smaller, homelike building. Both are licensed under N.J.A.C. 8:36 and surveyed by the Department of Health.

New Jersey's two signatures — medication and managed risk

- **Medication administration by delegation:** this is where New Jersey differs from states that allow only “assistance with self-administration.” Trained personal care assistants and certified medication aides may ADMINISTER medications under the delegation and supervision of a registered professional nurse. That model lets a well-run home meet significant medication needs — but it puts real weight on your RN's delegation, training, and oversight.
- **Aging in place through managed risk:** when a resident wants to make a choice that carries risk, you don't simply say no or discharge — you document the informed choice and your mitigation in a written managed-risk agreement, honoring autonomy while protecting health and safety. This is the concept that makes aging in place work.

Key terms, in plain English. ALR = Assisted Living Residence (apartment-style). CPCH = Comprehensive Personal Care Home (homelike, smaller). ALP = Assisted Living Program (services in subsidized housing). N.J.A.C. 8:36 = the licensing rules. CON = Certificate of Need (the first gate). DOH = NJ Department of Health (licenses you). DCA = Dept. of Community Affairs (reviews construction plans). MLTSS = Managed Long Term Services and Supports (Medicaid home/assisted-living services). PNA = Personal Needs Allowance (\$140.15/month the MLTSS resident keeps). RN = registered professional nurse (delegates medication). Managed risk = the written informed-choice agreement. NPI = National Provider Identifier.

Why this business — and why now

New Jersey is a high-cost, high-demand market for assisted living: the older-adult population is growing, families overwhelmingly want to keep a loved one in a homelike setting rather than a nursing home, and hospitals and skilled nursing facilities constantly need somewhere to step people down to. New Jersey's MLTSS also has a crucial advantage over many states — no enrollment cap and no waiting list — so a qualifying resident can actually get services, which you can honestly promise referral sources. A well-run home that ages residents in place and keeps its census strong is a durable business.

Be clear-eyed about the model choice. The single most common — and expensive — mistake new operators make is defaulting to an apartment-style ALR when a homelike CPCH would meet their plan at far lower cost. An ALR usually means new construction or a substantial conversion; a CPCH can start in a more residential building. Choose the model that fits your capital and your plan, not the one that sounds grander.

PART 2

Who Regulates You — and the Order of the Gates

In short: *The Department of Health licenses and surveys you, but you pass through the gates in order: Certificate of Need first, then construction plan review through Community Affairs, then the license. Separate bodies license your administrator and take abuse reports.*

NJ Department of Health — Division of Long-Term Care Systems (your licensor)

The DOH's Office of Certificate of Need & Healthcare Facility Licensure issues your CON and your license and surveys you after you open. This is your primary regulator under N.J.A.C. 8:36 and N.J.S.A. 26:2H (the Health Care Facilities Planning Act).

Dept. of Community Affairs — Health Care Plan Review (your building)

New Jersey separates construction plan review from health licensing. Any ALR or CPCH with a construction program must submit plans to Health Care Plan Review in the DCA, Division of Codes and Standards, for approval BEFORE construction begins — and you obtain a certificate of occupancy from the municipality before you occupy.

The administrator licensing board, and NJ FamilyCare / MLTSS

Your assisted living administrator is licensed through the state's licensing board (confirm the current qualification and route before you hire). NJ FamilyCare / the Division of Medical Assistance delivers Medicaid services through MLTSS and the managed care organizations — your services payer for qualifying residents.

Adult Protective Services & reportable events

You are a mandated reporter. Suspected abuse, neglect, or exploitation is reported through the county ADRC / APS (statewide 1-877-222-3737) and the DOH complaint hotline, and you report reportable events to the Department within required timeframes.

What this means for you: the order matters. CON, then plan review, then license — skip or reorder them and you invite the most expensive kind of rework. Confirm the current CON pathway with the DOH early (Part 5), and don't build until DCA approves your plans (Part 6).

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In New Jersey you form the LLC through the Division of Revenue & Enterprise Services, then complete the NJ Business Registration (NJ-REG) that gives you your tax and employer IDs. New Jersey will also evaluate the proposed operator's licensing track record — in NJ and other states — as part of licensure, so your ownership and management history matters from the start. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

1. **File your Certificate of Formation.** Through the Division of Revenue at nj.gov/treasury/revenue — the filing fee is \$125.
2. **Complete the NJ-REG business registration.** Required after you form the entity, to get your NJ tax and employer identification and to hire employees.
3. **Name a registered agent and adopt an Operating Agreement.** The agent (which can be you) receives legal mail at a NJ address; the Operating Agreement sets ownership and management.
4. **Calendar the annual report.** NJ LLCs file an annual report — \$75 — each year with the Division of Revenue to stay in good standing.

No newspaper publication in New Jersey. Unlike New York, NJ has no LLC publication requirement — forming the entity is a straightforward filing plus the NJ-REG registration.

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

1. **Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
2. **Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
3. **Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
4. **Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to enroll to participate in Medicaid MLTSS so you can bill for the services eligible residents receive. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

1. **Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
2. **Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the assisted-living / residential-care taxonomy that fits your facility — confirm the exact one NJ FamilyCare / MLTSS wants. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of NJ branches — TD Bank, Bank of America, Wells Fargo, PNC, or a NJ option like Provident or Columbia, or a local credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

An assisted living home houses and cares for vulnerable residents around the clock, so you need coverage built for a licensed residential care setting — and your lenders, DOH, and MLTSS relationships will expect proof:

Coverage	What it protects against	Must-have?
General & Professional Liability	Injury or property damage, plus claims about the CARE (personal care, medication, supervision)	Yes — foundational
Abuse & Molestation	Allegations of abuse of a resident — general liability EXCLUDES this	Yes — high exposure
Property & Business Interruption	Fire and damage to the building, plus lost income	Yes
Directors & Officers / Management	Regulatory, employment, and governance claims	Recommended
Workers' Compensation & Disability	Staff injuries; NJ also mandates temporary disability coverage	Required in NJ for employees
Non-Owned & Hired Auto	Staff driving residents or errands	Recommended

New Jersey requires disability coverage too. On top of workers' comp, NJ employers must carry Temporary Disability Insurance. Your payroll provider or broker sets this up — factor it into your labor cost.

Real places to get it

Buy through an independent agent or broker who writes SENIOR-LIVING / assisted-living programs — this is a specialty niche. Carriers and programs active in assisted living include Philadelphia Insurance (PHLY), CNA, The Hartford, Great American, Markel, and senior-living specialty programs. Find a local NJ agent at [trustedchoice.com](https://www.trustedchoice.com) and ask specifically for an assisted living / long-term care program. Confirm the carrier writes NJ assisted living before you rely on a quote.

What to ask the insurance agent

“Do you write NJ assisted living residences / CPHs — which carriers, and what's the minimum premium?”

“Is abuse & molestation included, at what limit, and is it a sub-limit or a full limit?”

“Does professional liability cover our RN-delegated medication-administration model?”

“What limits do my lender, DOH, and the MLTSS managed care organizations expect?”

“Can you set up NJ workers' comp AND temporary disability coverage?”

PART 4

Choose Your Model — ALR or CPCH

In short: *This choice drives your building and your capital. An ALR requires apartment-style units; a CPCH allows homelike bedrooms of up to two residents. Both deliver the same services and both must meet N.J.A.C. 8:36.*

Start here, because the model decides how much you have to build. An ALR requires apartment-style units — each with its own bath and kitchenette — which usually means new construction or a substantial conversion and more capital. A CPCH lets you serve residents in bedrooms (no more than two residents per unit, each with a lockable door) in a more homelike building, which is why smaller operators, and conversions of former residential health care facilities or boarding homes, often begin here. Both deliver the same assisted living services and both must meet N.J.A.C. 8:36; the difference is the setting and the cost to build it.

The smaller-operator path. *If you're a first-time operator or converting an existing residential building, the CPCH is usually the realistic entry point — it meets the same care standard at far lower construction cost. Many operators start with a CPCH and grow.*

PART 5

The Certificate of Need — Your First Gate

In short: *Unlike many states, New Jersey requires a Certificate of Need before you can be licensed. The good news: assisted living qualifies for an expedited CON review that does not go before the State Health Planning Board.*

The CON is the state's way of approving that the service is appropriate before the license application proceeds. For assisted living, New Jersey law permits an EXPEDITED certificate-of-need review — less intensive than the full review, and it does not go before the State Health Planning Board. Treat the CON as the gate you pass through first.

- 1. Confirm the current CON pathway and forms.** Contact the DOH's Certificate of Need and Licensing office (Part 12) to confirm the expedited assisted living pathway, the forms, and the fee before you file.
- 2. Understand the commitments that attach.** Your CON approval can carry commitments — notably the Medicaid-access commitment (Part 10) — that you will be held to after you open. Know them going in.
- 3. Get the CON before you commit capital.** Do not sign a lease or start construction assuming you can license without the CON — it comes first, by law.

PART 6

Plan Review & the Physical Plant

In short: *New Jersey separates construction plan review from health licensing. Any ALR or CPCH with a construction program must get plans approved by Community Affairs BEFORE building — and a certificate of occupancy before residents move in.*

Submit your construction plans to Health Care Plan Review in the DCA, Division of Codes and Standards, for review and approval before construction begins — whether or not a CON is required. Before occupying a new or renovated building, obtain a certificate of occupancy from the municipality and provide the required releases to the DOH.

Plan the physical plant around your model

- **An ALR** must provide apartment-style units meeting the space, privacy, bath, and kitchenette requirements.
- **A CPCH** must provide residential units housing no more than two residents with a lockable door.
- **Both** must meet life-safety and fire requirements, provide accessible common and dining areas, and support aging in place.

Get plan review right early. Approving plans through DCA before you build prevents the most expensive kind of rework — the kind you discover after you've built. Involve an architect familiar with health-care-facility codes from the start.

PART 7

The License Application

In short: After your CON, you apply to the Commissioner of Health on the prescribed forms, submit your admission agreement for review, pay \$1,500 plus \$15 per licensed bed, and pass a pre-licensure survey before residents move in.

- 1. Apply on the DOH's prescribed forms.** Obtained from the Office of Certificate of Need and Healthcare Facility Licensure, with documentation of your operator, ownership, and physical plant.
- 2. Submit your admission agreement for review.** DOH confirms it doesn't conflict with the rules or your CON conditions. Apply for the maximum number of beds available in your residential units.
- 3. Pay the licensing fee.** A nonrefundable \$1,500 application fee plus \$15.00 per licensed bed — and the same fee applies at each annual renewal, subject to the statutory caps. Confirm the current fee before you file.
- 4. Pass the pre-licensure survey.** Have your policies adopted, your administrator and RN in place, your medication-delegation system established, and your resident records and forms ready when the surveyors arrive.

What this means for you: the pre-licensure survey isn't a formality — the surveyors confirm your building, staffing, and systems are ready before residents move in. The operators who pass the first time show up with the whole operation actually built: policies adopted, administrator and RN hired, medication aides trained and delegated, and the assessment and service-plan forms ready to demonstrate.

PART 8

Leadership, Staffing & the Medication Model

In short: Two roles anchor the operation: a licensed assisted living administrator and a registered professional nurse. The RN delegates and supervises medication administration by trained aides — New Jersey's signature model — and you staff 24 hours with awake overnight coverage.

8.1 The two anchor roles

- **A licensed assisted living administrator:** responsible for day-to-day management, licensed through the state's board. Confirm the current qualification and licensing route before you hire.
- **A registered professional nurse (RN):** New Jersey expects an RN presence sufficient to oversee residents' health care plans and — critically — to delegate and supervise medication administration. Build your plan around a full-time RN.

8.2 The medication-administration-by-delegation model

Trained, unlicensed staff — personal care assistants who have completed the required nurse-aide training, and certified medication aides who have completed medication-aide training — may administer medications to residents UNDER the delegation and supervision of the registered professional nurse. That model lets a well-run home meet significant medication needs, but it depends on real RN capacity: thin nursing coverage is both a compliance risk and a safety risk. Build your staffing around a full-time RN, a trained medication-aide workforce, and the criminal-background screening New Jersey requires for health care facility staff.

8.3 Staffing to census and acuity

Plan enough qualified staff to make assisted living services and supervision available 24 hours a day, with awake overnight coverage sufficient for your residents' needs. Your staffing is not a fixed number — it rises with census and acuity, which is the practical cost of aging in place. Model your labor as a variable that grows with your residents' needs, not a flat line.

8.4 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 9

Policies, Assessment & Managed Risk

In short: Before you open you adopt the written policies and the assessment-and-service-planning process at the heart of the rules — and New Jersey's managed-risk agreement, the tool that makes aging in place work.

9.1 Assessment and service planning

Each resident is assessed on admission, periodically, and on a change in condition, with registered-professional-nurse involvement. From that assessment you build an individualized service plan and a health care plan that say exactly what services, supervision, and health-related care you will provide. This assessment-to-plan cycle is the clinical and the compliance backbone of the home.

9.2 Managed risk — how aging in place works

When a resident wants to make a choice that carries risk — a mobility choice, a dietary choice, a decision to decline a recommended service — you do not simply say no or discharge. You document the resident's informed choice and the steps you'll take to reduce the risk in a written managed-risk agreement, honoring autonomy while protecting health and safety.

Managed risk is not a waiver. A managed-risk agreement documents an informed choice and YOUR mitigation — it does not excuse failing to meet a resident's assessed needs. Surveyors look for real assessment, real service plans, and real mitigation, not a signature that waves away your responsibility.

9.3 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 10

How You Get Paid — Room & Board, Services & the Medicaid Commitment

In short: Two clean halves: room and board is paid privately (resident income + SSI); services are paid privately or by Medicaid MLTSS for qualifying residents. And licensure carries a Medicaid-access commitment you plan for from day one.

10.1 The two halves — room and board, and services

Room and board — the rent, meals, and housekeeping — is paid privately, from the resident's own income and savings, or supplemented by SSI where they qualify. Medicaid NEVER pays room and board in assisted living. Services — the personal care, supervision, medication administration, and health-related assistance — are paid either privately or, for residents who qualify, by New Jersey Medicaid through MLTSS under NJ FamilyCare.

***NJ MLTSS has no cap and no waitlist.** This is a real advantage over many states: a resident who meets the clinical and financial tests receives services — they're not competing for a limited pool of slots. You can honestly tell referral sources that a qualifying resident can get services.*

10.2 The MLTSS arithmetic (be precise with families)

When a resident is on MLTSS, the room-and-board math is specific: the resident keeps a personal needs allowance of \$140.15 per month and turns the rest of their monthly income toward room and board, with the balance covered privately or by SSI. Understanding this split — Medicaid pays services, the resident's income pays room and board, and \$140.15 stays with the resident — is the key to pricing your home and explaining costs to families honestly.

10.3 New Jersey assisted living funding — the honest table

Item	Amount / rule (verify current figures)
What MLTSS covers	Assisted living services (personal care, supervision, medication administration, health-related) — NEVER room and board
MLTSS caps / waitlist	No enrollment cap and no waiting list — qualifying applicants receive services
Clinical eligibility	Nursing-facility level of care — hands-on help, or supervision for cognitive impairment, with 3+ activities of daily living
Income limit (2026)	Up to about \$2,982/month (300% of the SSI federal benefit rate); a Qualified Income Trust is used if income is over the limit
Asset limit (2026)	Under \$2,000 in countable resources for a single applicant (spousal rules differ)
Personal needs allowance	\$140.15/month kept by the MLTSS resident; the remainder of income applied to room and board
Room & board	Private pay from resident income, supplemented by SSI where the resident qualifies — not Medicaid
Private-pay average (2026)	About \$8,234/month statewide for assisted living (varies widely by county, unit, and care level)
Licensing fee	\$1,500 application + \$15.00 per licensed bed, at application and each annual renewal (subject to statutory caps)

10.4 The Medicaid-access commitment

New Jersey does not let assisted living serve only private-pay residents. Licensure carries a Medicaid-access commitment, and you plan for it from day one. An assisted living residence commits that, within 36 months of licensure, at least 20 percent of its residents (measured on a resident-day basis) will be people with nursing-home level-of-care needs; a CPOCH has a Medicaid-access requirement of its own. Practically, this means you enroll to participate in MLTSS, build relationships with the managed care organizations, and admit and retain Medicaid-eligible residents as part of your census — not as an afterthought.

***Track it from the start.** A home that ignores the Medicaid-access commitment until month 30 can find itself scrambling; a home that plans for it prices and markets accordingly and treats MLTSS participation as a core part of the business. Track your resident mix against your commitment on a regular cycle, and document it.*

PART 11

What It Costs, and Finding Your Residents

In short: *Startup costs vary enormously between a new apartment-style ALR and a homelike CPCH conversion, so treat them as planning categories. Residents come from the people families turn to — discharge planners, skilled nursing, home health, and the county ADRC.*

11.1 What it costs to start — planning categories

The only fixed, verified state figure is the licensing fee; everything else depends on your building, county, and model.

- **Certificate of need & licensing:** the \$1,500 + \$15/bed fee at application and each renewal, plus any professional help with the CON and application.
- **Real estate & construction:** the largest and most variable cost — an apartment-style ALR costs far more to build or convert than a homelike CPCH.
- **Plan review, permits, fire safety & certificate of occupancy:** architectural and code work through DCA and your municipality.
- **Leadership & staffing before opening:** recruiting a licensed administrator and an RN, and training your medication-aide workforce.
- **Insurance & working capital:** liability, property, and workers' comp/disability — plus several months of operating reserve to carry payroll while census builds.

11.2 Finding residents

Referrals in assisted living come from the people families turn to when a loved one can no longer manage alone: hospital discharge planners and social workers, skilled nursing facilities managing step-downs, home health and hospice agencies, and the county Aging and Disability Resource Connection (ADRC), where many families start. Because New Jersey's MLTSS has no waitlist, you can honestly tell referral sources that a qualifying resident can get services — a real advantage worth making known.

Advertise truthfully. *New Jersey allows only licensed assisted living residences and CPCHs to advertise as “assisted living.” Describe your license truthfully, never claim services you don't provide, and let your managed-risk, aging-in-place philosophy — the promise that a resident can stay as needs change — be the story families remember.*

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 12

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact and verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here. Statewide contacts are the same for everyone; Medicaid eligibility and permits are county/city.

Licensing, plan review & the administrator

Who	Contact	What to verify / ask
DOH — CON & Healthcare Facility Licensure	nj.gov/health (Long-Term Care Systems)	The expedited CON pathway; the license forms & fee (\$1,500 + \$15/bed); pre-licensure survey
DCA — Health Care Plan Review	nj.gov/dca (Codes & Standards)	Construction plan review before you build; certificate of occupancy
Assisted living administrator licensing board	the NJ licensing board	The current administrator qualification and licensing route
DOH complaint / reportable events	800-792-9770 (verify)	Reportable-event timeframes and how to report

Business, insurance & billing IDs

Who	Contact	What to verify / ask
NJ Division of Revenue & Enterprise Services	nj.gov/treasury/revenue	Forming your LLC (\$125); NJ-REG; the \$75 annual report
IRS	irs.gov (EIN Assistant)	Getting your EIN (free)
NPPEs / CMS	npes.cms.hhs.gov	Your Type 2 NPI (for MLTSS billing)
Insurance agent (senior-living program)	trustedchoice.com ; carriers in Part 3.5	Liability + abuse + property + workers' comp + NJ disability

Medicaid MLTSS & referrals (county-level)

Who	Contact	What to verify / ask
County Board of Social Services	your county board	MLTSS financial eligibility (income \$2,982 / assets \$2,000)
County ADRC / Division of Senior Services	1-877-222-3737 (statewide)	MLTSS assessment referral; where families start
NJ FamilyCare MCOs	Aetna, Fidelis, Horizon, UnitedHealthcare, Wellpoint	Enrolling to participate in MLTSS; contracting to bill services
Adult Protective Services	route via ADRC 1-877-222-3737	Reporting suspected abuse, neglect, or exploitation

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I have to take Medicaid, or can I run private-pay only?

You cannot run private-pay only. New Jersey licensure carries a Medicaid-access commitment — an ALR commits that within 36 months at least 20% of residents (on a resident-day basis) will have nursing-home level-of-care needs, and a CPCH has its own requirement. Plan MLTSS participation and your resident mix from day one, not at month 30.

Should I open an ALR or a CPCH?

If you're a first-time operator or converting an existing residential building, the CPCH is usually the realistic entry point — it meets the same N.J.A.C. 8:36 care standard in a homelike setting at far lower construction cost than apartment-style ALR units. Many operators start with a CPCH and grow. Defaulting to an ALR is the most common over-building mistake.

Does Medicaid pay for room and board?

No — Medicaid MLTSS pays for SERVICES only (personal care, supervision, medication administration, health-related care), never room and board. Room and board is paid from the resident's own income and savings, supplemented by SSI where they qualify. The MLTSS resident keeps a \$140.15/month personal needs allowance and applies the rest of their income to room and board.

What makes New Jersey's medication model different?

Here, trained unlicensed staff may ADMINISTER medications — not merely assist with self-administration — under the delegation and supervision of a registered professional nurse. That lets a well-run home meet significant medication needs, but it depends on real, full-time RN capacity for delegation, training, and oversight. Thin nursing coverage is both a compliance and a safety risk.

You can do this

Opening a New Jersey assisted living home is a real capital-and-compliance undertaking — but it's a walkable path, and the demand is deep. Choose the model that fits your capital (often a CPCH to start), pass the gates in order (CON, then plan review, then license), build the operation around a licensed administrator and a full-time RN, honor aging in place through managed risk, and plan your Medicaid-access commitment from day one. Verify each fee and contact as you go, and lean on the people in Part 12.

Reminder. Prepared for PolicyReady.org and grounded in N.J.A.C. 8:36, N.J.S.A. 26:2H, and current NJ FamilyCare / MLTSS information as of 2026. This is general information, not legal, tax, or clinical advice. Requirements, fees, rates, and contacts change — verify each with the Department of Health, the Department of Community Affairs, NJ

FamilyCare, and qualified counsel before you rely on it. Confirm the current CON pathway, the licensing fee, and the MLTSS figures before you rely on them.